

SOP / Clinical Alert Request Form

Use this form to tell the ERP team what alerts your protocol needs. Write in clinical language — the team converts it into the system's rules. One form per protocol.

HOW TO FILL THIS IN

1. Complete Part 1 and Part 2 once, for the whole protocol.
2. Complete one ALERT REQUEST card for each separate alert you want. Photocopy that page as many times as you need.
3. In each card, question 6 asks WHEN the alert should fire. Tick only ONE of options A–G, and fill in just that option.
4. Appendix A lists what the system already measures. If what you need isn't there, write it in anyway — we'll tell you whether it needs development first.
5. Leave every shaded **"FOR ERP TEAM USE"** box blank.

PART 1 — ABOUT THIS SOP

Protocol / SOP name

Document number Version Dated

What is this protocol for, in one line?

Prepared by (name) Designation / department

PART 2 — WHICH PATIENTS DOES IT APPLY TO?

These answers act as a gate. Unless a patient matches ALL of them, none of the alerts in this form will fire for that patient.

a. Which centre(s)?

☐ All centres

b. Which patients?

☐ Admitted (IPD) patients only ☐ OPD patients only ☐ All patients

c. Only for particular diagnoses?

- ☐ No — applies to every diagnosis
- ☐ Yes — only these diagnoses (name them, with the ICD code if you know it):

.....

.....

d. Any other limit on which patients this applies to?

e.g. age above or below a number, one gender only, a particular admission type or priority.

.....

.....

PART 3 — ALERT REQUESTS (photocopy this page for each additional alert)

ALERT REQUEST 1

Short name for this alert

1. What should the alert say to whoever receives it?

Write it as you would want to read it. You may use {patient.name} and {field.value} — the system fills those in.

2. How urgent is it?

☐ Low — note it, no rush ☐ Medium — review today ☐ High — review now ☐ Critical — act immediately

3. Who must be told?

Role(s) / designation(s)

☐ The patient's admission doctor ☐ The patient's admission psychologist

Specific people by name (if any)

4. What should they do about it?

5. Protocol / document section this comes from

6. WHEN should it fire? Tick ONE option (A–G) below and fill in only that one.

☐ **A. A reading or score crosses a limit** fires the moment it is saved

Which assessment or reading (Appendix A)

Fires when the value is:

☐ above ☐ at or above ☐ below ☐ at or below ☐ exactly equal to
this value or between and

☐ **B. Something was not done by a deadline** one-off check

What must be recorded

Within hours of admission. Allow hours grace.

☐ **C. Not being done often enough** one line per stage; leave TO DAY blank for "onwards"

What must be recorded

HOW MANY TIMES	PER (day/week/month)	FROM DAY	TO DAY	GRACE (hours)

☐ **D. The patient has now been admitted a certain number of days** one-time milestone

Alert once the admission is older than days.

☐ **E. Due on particular days of the admission** e.g. day 1, 3 and 7

Due on admission day(s)

☐ ...and then every day after the last day listed, until discharge

☐ **F. Due every so many hours, continuously** admission until discharge

Every hours. Allow hours grace.

☐ **G. The score has stayed low — suggest stopping monitoring** fires once; resets if a score rises

Which assessment

Score stays below on assessments in a row.

7. If this is a LAB result, complete this box instead of the value in A

Which test (Appendix A) — or write ANY for any flagged test

☐ flagged severity is or worse (Low / Medium / High / Very High)

☐ the value reaches times the upper normal limit, or more

8. Should the reminder stop once the patient is stable?

☐ Yes — stop after assessments all below (leave blank to keep reminding)

FOR ERP TEAM USE — LEAVE BLANK

Model / field

Operator / value

Trigger / schedule

Rule + block name

ALERT REQUEST 2

Short name for this alert

1. What should the alert say to whoever receives it?

Write it as you would want to read it. You may use {patient.name} and {field.value} — the system fills those in.
.....
.....

2. How urgent is it?

☐ Low — note it, no rush ☐ Medium — review today ☐ High — review now ☐ Critical — act immediately

3. Who must be told?

Role(s) / designation(s)

☐ The patient's admission doctor ☐ The patient's admission psychologist

Specific people by name (if any)

4. What should they do about it?

.....
.....

5. Protocol / document section this comes from

6. WHEN should it fire? Tick ONE option (A–G) below and fill in only that one.

☐ **A. A reading or score crosses a limit** fires the moment it is saved

Which assessment or reading (Appendix A)

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☐ above ☐ at or above ☐ below ☐ at or below ☐ exactly equal to
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☐ **C. Not being done often enough** one line per stage; leave TO DAY blank for "onwards"

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☐ ...and then every day after the last day listed, until discharge

☐ **F. Due every so many hours, continuously** admission until discharge

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☐ **G. The score has stayed low — suggest stopping monitoring** fires once; resets if a score rises

Which assessment

Score stays below on assessments in a row.

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Which test (Appendix A) — or write ANY for any flagged test

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FOR ERP TEAM USE — LEAVE BLANK

Model / field

Operator / value

Trigger / schedule

Rule + block name

PART 4 — MEDICINES TO SUGGEST (optional)

Medicines listed here appear as suggestions on the prescription screen for any patient this protocol applies to. They are suggestions only — nothing is ever prescribed automatically.

[illegible]

PART 5 — ANYTHING ELSE WE SHOULD KNOW

Escalation steps, exceptions, anything that didn't fit the cards above, or alerts you're unsure the system can do.

PART 6 — SIGN-OFF

Clinician _____ Signature _____ Date _____	Approved by _____ Signature _____ Date _____
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FOR ERP TEAM USE — LEAVE BLANK

Received on	SOP rule name
Configured by	Blocks created

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Configured by	Blocks created

WORKED EXAMPLE — how a filled-in card looks

For reference only. Do not fill this page in.

ALERT REQUEST 1 (example)

Short name	CIWA-Ar high — urgent review
1. Message	CIWA-Ar {field.value} for {patient.name} — severe alcohol withdrawal. Review immediately.
2. Urgency	High
3. Who must be told	Nursing, On-call physician + the patient's admission doctor
4. What they should do	Assess at bedside within 30 minutes; give benzodiazepine per protocol; inform the duty consultant.
5. Protocol section	JRC/DA/AWP/002 §VIII.C
6. When should it fire	Option A ticked — a reading crosses a limit
Which assessment	CIWA-Ar
Fires when value is	"at or above" ticked, this value: 15
7. Lab box	not applicable
8. Stop when stable	left blank

FOR ERP TEAM USE — filled in after configuring

Model / field	ciwaTest · systemTotalScore
Operator / value	GREATER_THAN_OR_EQUAL · 15
Trigger / schedule	IMMEDIATE · —

A second example — an alert for something that was not done

"Option C ticked. What must be recorded: Counselling session. 2 times per week, from day 5 to day 30, grace 24 hours." That produces one alert whenever a patient at that stage of admission has had fewer than two counselling sessions in the week.

A third — a length-of-stay milestone

"Option D ticked. Alert once the admission is older than 30 days." Used for review meetings, insurance checkpoints, and anything else that is due because of how long the patient has been with us rather than because of a reading.

Common mistakes worth avoiding

- Ticking more than one of A–G on the same card. If an alert genuinely has two conditions that must BOTH hold, say so in Part 5 — we can build that, but it needs describing in words.
- Writing "urgent" in the message but choosing Low urgency. The urgency you tick is what decides how the alert is presented and who chases it.
- Leaving question 3 blank. An alert with nobody to receive it is never seen.
- Naming a reading we don't record yet. Check Appendix A first; if it isn't there, it needs development before the alert can exist.

APPENDIX A — WHAT THE SYSTEM ALREADY MEASURES

Anything listed here can be used in a rule today. If what you need is missing, write it into the card anyway — we will tell you whether it needs development first.

Vital signs (Vital Sign chart)

Blood pressure — systolic · Blood pressure — diastolic · Pulse · Temperature · Respiration rate · SpO2 · Blood sugar · Weight · CNS · CVS · RS · PA

Withdrawal, sedation and risk scales (total score unless noted)

CIWA-Ar · COWS · Ramsay Sedation · Morse Fall Risk · Glasgow Coma Scale (total, and eye / verbal / motor separately) · AUDIT

Psychiatric rating scales (total score unless noted)

Y-BOCS · C-SSRS (total, and ideation / behaviour subscales separately) · YMRS · HAM-A · HAM-D · PANSS (total, and severity band) · MMSE (total, and interpretation) · MPQ · ACDS · CGI-S · Rorschach · generic clinical test

Laboratory results (by flagged severity, or as a multiple of the upper normal limit)

ALT / SGPT · AST / SGOT · ALP · Total bilirubin · Albumin · Ammonia · INR / PT-INR · Creatinine · BUN · eGFR · Sodium · Potassium · Magnesium · Phosphate · Calcium · Glucose (FBS / RBS) · HbA1c · Haemoglobin · WBC / TLC · Platelets · CPK / CK

Patient and admission details

Age · Gender · Currently admitted · Discharged · Admission date (this is what “admitted N days” rules use) · Provisional diagnosis · Patient type (IPD / OPD) · Priority · Whether the admission, consent, discharge or undertaking form has been completed

Clinical records — usable for their content, and for “was this recorded at all?”

Prescription (doctor's notes, diagnosis) · Clinical Note (complaints, observations, diagnosis, notes) · Mental Status Examination (chief complaints, mood, thought content and process, perception, memory, insight, judgment, remarks, observation) · Counselling Note (conclusion, objective, short- and long-term goals, homework, next session date) · Detailed Admission (physical examination — CNS, CVS, pulse, BP, RS, PA; provisional diagnosis; age; blood group)

On “was this recorded at all?”

Every record type above can also be checked simply for existence. That is what options B, C, E and F rely on — the alert fires because nothing was recorded in the expected window, not because a value was wrong.

Two limits worth knowing before you write

- Threshold alerts (option A) fire immediately, the moment the reading is saved.
- The scheduled checks (options B–F) run twice a day, at about 02:00 and 14:00. An alert for a missed window therefore appears at the next run, not the minute the deadline passes. If something needs to be caught faster than that, say so in Part 5.

APPENDIX B — WHAT THE ALERT LOOKS LIKE WHEN IT ARRIVES

Alerts appear in the Alerts screen for everyone you named in question 3. Each one shows:

- the urgency you chose, as a coloured band
- the message you wrote in question 1, with the patient's name and the actual reading filled in
- the action you wrote in question 4
- the protocol section from question 5
- which patient it concerns, and the reading that triggered it

Recipients can add notes to an alert as they work on it, and mark it resolved once the action has been taken. Both are recorded against the person's name and the time. Alerts can also be exported for audit.

The same alert is not repeated for the same patient and the same window, so a missed record produces one alert rather than one per check.

Choosing an urgency

Low — worth knowing; no action needed today.

Medium — should be reviewed during the shift.

High — needs attention now.

Critical — patient safety issue; act immediately.

Please use Critical sparingly. If everything is critical, nothing is.

If you're not sure

If anything on this form is unclear, or you are unsure whether the system can do what you need, send the form back with a note in Part 5 rather than leaving it blank. It is easier for us to answer a question than to guess at an intention.

FOR ERP TEAM USE — WHERE THIS GETS BUILT

Part 2 becomes the rule's Satisfying Criteria, plus its centres. Each ALERT REQUEST card becomes one Target Block on that rule. Part 4 becomes the Suggested Medicines. Option A and question 7 are immediate-phase conditions; options B–G drive the delayed-phase schedule.